Program decisions integrate several assay readouts into one go/no-go

Across 7 such advancement evaluations (230 runs) models pass only 35%; the same call can fail by advancing too much or too little.

CORRECT

Advance one compound, or none

Colorectal-cancer organoid program. Inputs: primary screen, mechanism image, tox counterscreen, DMPK exposure. Advance a compound only if the desired mechanism, an adequate safety margin, and covering exposure all hold.

DATA-SUPPORTED CALL

Advance CMP-05, the one compound that satisfies all three criteria together.

MOSTLY CORRECT (82% PASS)

The tempting shortcut, ranking by potency or by total exposure, advances the wrong compound.

WRONG SYNTHESIS

Pick the next decision-gate models

NSCLC mTOR + WEE1 combination. The gate set must span models with confirmed multi-endpoint support, the strongest responder still lacking confirmation, and the strongest responder outside the primary genotype.

DATA-SUPPORTED CALL

Build the gate on convergent multi-endpoint support (e.g. PDX239 for the unconfirmed slot).

COMMON FAILURE (27% PASS)

Rank on a single synergy metric and nominate an artifact model (H2009), so the wrong models gate the program.

FALSE-GO

Advance only if safe and active

Blinded intestinal multiplex-IF package. A candidate may advance only with paired evidence of activity and healthy-tissue sparing.

DATA-SUPPORTED CALL

No candidate advances; the paired activity-and-safety evidence fails for every candidate.

COMMON FAILURE (36% PASS)

Advance on activity alone or a relative tumour-vs-healthy index, pushing an unsafe candidate forward.